

Reports of Wives of Alcoholics of Effects of LSD-25 Treatment of Their Husbands

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THOUGH ALCOHOLICS have been treated with lysergic acid diethylamide (LSD-25) and similar psychodelic substances now for over a decade and many papers have reported the results of such treatment,¹⁻¹⁵ considerable controversy remains as to what can be accomplished by these means. The results reported have varied from no change to very encouraging; moreover, while investigators have frequently reported immediate and remarkable improvement in attitudes and behavior, it has also been noted that in the long run such gains have all too often disappeared.

In the present paper we hope to throw some light on the problem of differences in immediate and long-run changes in alcoholics as a result of LSD-25 treatment, and also to suggest how, in some cases, early gains might be better consolidated.

In general, information about the results of LSD treatment has been derived from studies of the treated persons themselves. We shall report here information derived from another source, the wife of the alcoholic treated with LSD.

This investigation was undertaken in the course of following up several groups of alcoholics who had been administered LSD over a period of years. We began to hear frequent and provocative references by the married alcoholics to what appeared to be secondary effects of the drug—that is to say, effects on the wives of the treated men. Some reported remarkably favorable changes

in their wives; on the other hand, some of these secondary effects appeared rather unpredictable. Thus, while in our view of the matter the effect on the husband may have been good, it appeared that his wife would not necessarily see things in this way. For instance, one husband reported that his wife was puzzled and dismayed because, she said, he no longer acted like an adolescent, another that his wife was annoyed by his new interest and involvement in community activities. We felt that such reactions on the part of their wives might undermine the effectiveness of the treatment the husbands had received.

Accordingly, we decided to investigate, six months after our latest group had taken the drug, how the wives perceived the changes which had taken place in their husbands, how they felt they had reacted to these changes, and what problems had emerged as a result of the treatment. Unfortunately, it was then too late to obtain premeasures of the wives' perceptions of these matters prior to treatment, so that we were forced to use retrospective data. However, we felt that an exploratory study of this sort would provide useful information which might help us to plan a later LSD program for alcoholics in which we could incorporate their wives into the treatment activities from the beginning.

The wife of the alcoholic has long been an object of scrutiny; she has alternately been blamed for the illness,¹⁶⁻²⁴ exonerated,²⁵⁻²⁶ described as a symbiotic partner to the situation,²⁷ or as part of a double-bind dominating the domestic interaction.²⁸ While Jackson and Kogan have examined the overall pattern and phases of change forced upon

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wives by the circumstances of marriage to an alcoholic,²⁹ the problem of change of the wife in relation to her husband's treatment has received little attention, though it has been suggested that these women become more than ordinarily resistant to change of any sort.²⁹

Treatment with LSD-25 presents an unusual opportunity to examine the result of dramatic change in the alcoholic's attitudes and behavior upon his wife. In sufficient dosages, it produces a "blunderbuss" assault on the senses³⁰ which, when digested and focused with relevance to the patient's difficulty, often results in a profound conversion of both attitude and behavior for a variable period of time.

However, while the husbands typically left the treatment center markedly altered in terms of attitudes and behaviors, starry-eyed and full of enthusiasm, we felt that their long-embattled spouses might be less sanguine about what had taken place in treatment and what might take place in the future. It seemed important to us to find out what kinds of changes these ladies perceived in their husbands, whether they felt that they had also changed in response, and what problems, if any, they felt had emerged as a result of the treatment. In order to examine differences between the wives' perceptions of these matters where husbands had been treated with LSD and another, more traditional, method, we also interviewed the wives of our comparison group in the study under discussion.

Method

From April through July of 1963, 28 alcoholics, hospitalized at the New Jersey Neuro-Psychiatric Institute Treatment Unit, took part in a four-week program combining two doses of LSD-25 with specially structured group therapy. Progress of these patients was evaluated at three months, six months, and 12 months following discharge and compared with that of 34 patients who took part in only the standard six-week program of the unit.

The Earle Alcoholic Unit is a 40-bed cottage devoted exclusively to the treatment of chronic alcoholism, situated on the grounds of the New Jersey Neuro-Psychiatric Institute, Princeton, NJ. It is directed by a board psychiatrist and includes on its staff one half-time resident, two social workers, one half-time psychologist, and five attendants. The usual daily census is close to 30, and only alcoholics who will agree to a six-week stay and otherwise give evidences of good motivation are admitted. Overtly psychotic and obviously organic cases are excluded. The standard program includes group and individual therapy, educational programs including AA

discussion, movies, pastoral counseling, occupational and recreational therapy.

Admissions to the unit average five per week. One week all admissions who did not present serious physical or psychological defects were asked to volunteer for the LSD program and four were selected. The next week all admissions were asked to volunteer for the comparison group and four were selected. Thus, the LSD and comparison groups, while not randomly selected, were similar in composition, though the LSD group did not contain any members with marked physical or psychological problems.

In a previous paper we have described in detail the nature of the LSD program, the research procedures, the characteristics of the LSD and comparison groups, and the results of the evaluation.³¹

In the present study, begun at the six-month interval, individual interviews were conducted with as many as would agree to take part of the wives of the alcoholics in both the LSD and comparison groups. Eighteen members of the LSD group were married, living with their wives, and locatable, and of these, 15 wives consented to take part in the study. In the comparison group, 17 were married, living with their wives, and locatable, of whom 11 wives cooperated with the study. None of these women had had any previous contact with the hospital staff, and were solicited only at this time; the difference in numbers obtained for the two groups (referred to here as the LSD group and the comparison group) reflects greater willingness among the LSD wives to participate.

Both groups of wives studied included a range of nationalities and religions. They were all between 26 and 63 years of age, the LSD group having a median age of 37, the comparison group, 46. For all but four (LSD), it was a first marriage; the median length of marriage was 14 years. Excepting one from each group, all had children. The average educational level for both groups was 12 years, and the majority held full-time jobs. None of the women themselves drank more than "socially," and most only once in a while, if at all.

In the course of the interview, a three-part questionnaire was filled in covering points of time before the treatment of their husbands, the first few weeks following the end of treatment, and six months later. In every case but two, interviews were conducted in person either at the institute or in the subject's home; in one case in each group, a mailed questionnaire substituted for a personal interview. All interviews were tape-recorded and subsequently transcribed.

The first portion of the questionnaire was self-administered and contained questions regarding background characteristics of the wives, present drinking patterns of their husbands, and ratings of change in husbands in relation to treatment as perceived at the time of interviewing.

The second part of the questionnaire was also self-administered, consisting of an adjective checklist taken from Connor's list of adjectives³² selected by alcoholics on which they showed change in relation to therapy. From this list, five adjectives were deleted by us and 39 added, making 66 in all—those added representing words frequently used by alcoholics and others reported in the literature to describe how they changed in relation to LSD. The wives were asked to check those adjectives which

TABLE 1.—*Improvement in Their Husbands as Rated by the Wives of the LSD And Comparison Group Six Months After Treatment*

	LSD Group (n=15)						Comparison Group (n=11)					
	Not Given	Much Worse	Somewhat Worse	Not at all	Somewhat Better	Much Better	Not Given	Much Worse	Somewhat Worse	Not at all	Somewhat Better	Much Better
Overall Beneficial Effect	0	0	0	0	5	10	0	1	0	2	5	3
Benefit in Particular Areas												
1. Sobriety	1	0	0	2	3	9	1	0	2	2	2	4
2. Role of breadwinner	1	0	0	3	1	10	3	0	1	3	2	2
3. Getting along at work	5	0	0	2	3	5	3	0	0	5	3	0
4. Getting along with family	5	0	0	2	3	5	0	1	0	4	5	1
5. Getting along with children	5	0	0	2	2	6	2	1	0	5	1	2
6. Getting along with wife	3	0	0	2	2	8	1	1	0	3	4	2
7. Getting along socially	3	0	0	5	2	5	1	1	0	3	4	2

described first their husbands, and then themselves, before, a few weeks after, and six months after treatment.

In the third part of the questionnaire, the interviewer asked the wife a series of open-ended questions. These were all focused on areas of change, and the informants were encouraged to give range to their observations, their opinions, and their problems. Content analysis was done on this material.

Results

Part 1. The Self-Administered Questionnaire.—At the time of interviewing, ratings of husbands by wives showed that four in the LSD group were not drinking at all compared with one in the comparison group, while two of the LSD group were drinking daily as opposed to five in the comparison group. The remainder of each group had had at least one episode of intoxication.

The results of the wives' ratings of benefits derived by their husbands from treatment are shown in Table 1.

In terms of overall beneficial effect, ten wives in the LSD group checked that their husbands were much better, five checked that they were somewhat better, two checked no change, and one that her husband was much worse.

With regard to specific areas of benefit, the most marked improvement in the LSD husbands in the eyes of their wives appeared in the areas of sobriety, performance as breadwinners, and ability to get along with their wives. The least benefit accrued in ability to get along in other social relationships. For the comparison group, greatest benefit was noted in terms of sobriety, with some increase in ability to get along with

TABLE 2.—*Improvement in Husbands and Wives of LSD Group (N=15) and Comparison Group (N=11) Shortly After Treatment and Six Months Later Shown by Numbers of Adjectives (Total=66) Selected by Wives on Which Percent Frequency of Subjects Showing Characteristics in Each Group Has Changed For the Better*

% Group Showing Improvement	Number of Adjectives							
	Husbands				Wives			
	LSD Group		Comparison Group		LSD Group		Comparison Group	
	Shortly After	Six Mo Later	Shortly After	Six Mo Later	Shortly After	Six Mo Later	Shortly After	Six Mo Later
Over 60 %	11	0	0	0	5	0	0	0
40 %-59 %	29	9	7	1	11	7	5	0
20 %-39 %	17	26	26	14	19	20	16	7
0 %-19 %	6	28	30	42	20	32	39	49
0 %-20 %	3	3	1	9	11	7	5	9
Over -20 %	0	0	2	0	0	0	1	1

their wives, their families, and in other social relationships. In every area, however, the LSD husbands were seen to improve markedly more than those in the comparison group.

Part 2. The Adjective Checklist.—In this section of the questionnaire, 42 of the adjectives listed were positive and 24 were negative. In Table 2 are shown the numbers of adjectives selected by the wives to describe their husbands or themselves on which the groups showed improvement. (For the positive adjectives, there was a higher percentage of the group described as having the characteristic; for the negative adjectives, the percentage was lower.)

Although the number of adjectives on which a large percent of the LSD group changed suggests that a dramatic short-run improvement was seen by the wives of the LSD husbands, the long-run picture is less impressive. Yet, in spite of this drop-off, the wives' view of the improvement status of the LSD husbands remains, at the end of six months, as high as that indicated by the comparison group wives shortly after treatment. At six months, improvement seen among the comparison husbands has all but disappeared.

The wives in the LSD group also indicate, in the short run, more improvement in themselves than those in the comparison group; however, in both groups, this improvement is nowhere as great as that indicated for their husbands—the discrepancy being, of course, much more striking between the LSD spouses. With regard to sustained improvement, the wives in both groups are a conservative reflection of their husbands.

In the short run the husbands who had LSD showed improvement as far as their wives were concerned mostly in areas of overall reliability and in other characteristics which one would expect to lead to better rapport with their wives. Thus, over 60% were rated not only more responsible, truthful, dependable, and less wasting of money, but as more talkative and understanding, less critical of others, less resentful, changeable, or moody, as well as more accepting of themselves. Surprisingly, only 20% to 39% of the wives noticed a broadening of their husbands' interests—a typical reported result of LSD experience.

In the long run, 40% to 59% of these husbands were seen to remain more confident, more self-accepting and less resent-

ful, hostile, physically abusive, self-pitying, discouraged, reckless, and with less tendency to waste money.

The wives of the comparison group husbands found them to change most in terms of diminished negative traits. Thus, shortly after treatment, 40% to 59% were seen as more tolerant, and less easily hurt, dissatisfied, critical of others, hostile, quarrelsome, and discouraged.

After six months, improvement seen in husbands in this group at the 40% to 59% level had dwindled to less dependence.

Among the wives of the LSD patients, greatest change was noticed in terms of negative traits. Shortly after treatment, over 60% rated themselves less discouraged, irritable, bitter, resentful, and nervous. Forty percent to 59% of the group also described themselves as more hopeful, peace-loving, considerate, confident, and loving, following treatment. However, improvement was greatly diminished in this grouping at six months, though 40% to 59% rated themselves as less tense, bitter, self-pitying, irritable, moody, resentful, and discouraged.

Change in both positive and negative characteristics was noticed by 40% to 59% of the wives in the comparison group. They rated themselves shortly after treatment as more loving and hopeful, and less discouraged, dissatisfied, and bitter. At the six-month period, change was reduced; 20% to 30% felt themselves more truthful, considerate, and loving, and less tense, nagging, bitter, and worrying.

Part 3. The Focused Interview.—The wives were first asked whether they felt their husbands had changed for the better in any ways as a result of the treatment. All 15 wives of the LSD group responded affirmatively to this question, and ten of the 11 comparison group wives; only one of the latter responding negatively. One hundred and forty-four benefits were mentioned by the LSD wives, while the comparison group wives mentioned only 68; also the LSD wives were markedly more enthusiastic. The LSD wives most frequently mentioned changes in their husbands in terms of awareness, understanding, and tolerance of others. They also noted a greater sense of financial responsibility, better work adjustment, and an expansion of church, recreational, and social activities in their husbands. The comparison group wives most frequently men-

tioned better work adjustment, but also noted interpersonal changes as in the case of the LSD husbands.

The wives were then asked whether they had noted any negative changes as a result of the treatment. Twelve of the 15 LSD wives reported no negative changes. One reported that her husband was very depressed and irritable when he first came home but after three weeks he suddenly changed and became very tolerant, loving, easier to communicate with, etc. Another reported her husband seemed more remote from her after the treatment, drank heavily, and was more abusive and violent toward her and the children than before. However, shortly after this, this husband stopped drinking and the relationship became very good. The third wife who reported negative change said that her husband became more withdrawn from her and more uncommunicative than before, and their relationship worsened progressively. This man remained completely sober throughout the follow-up period but his marriage declined.

Only one of the comparison group reported negative change as a result of the treatment, and this was in terms of her husband's being ashamed to admit that he was an alcoholic. She felt the treatment had helped him admit it to himself but that he did not want others to know.

The wives were then asked whether any specific problems had arisen as a result of the treatment. Ten of the LSD wives reported no problems. One said that when her husband drank now his behavior was worse than before treatment, another that her husband was more aware of being no good. A third (the one mentioned before, whose marriage was deteriorating) reported her husband was more withdrawn from her than before, accused her of taking the children's love from him, and did not seem to want her love. Another wife complained that the cure seemed too pat and easy, and indeed her husband, in the beginning overconfident, shortly began to drink heavily. A fifth complained that another member of the LSD group with whom her husband had struck up a friendship had begun to drink and had started her husband drinking. This contagion was a regrettable aspect of the otherwise useful, cohesive group structure the LSD program had developed in the men.

Nine of the comparison group wives reported no problems as a result of treatment.

The remaining two said that their husbands were ashamed of having been in the hospital and did not wish the neighbors to know about it.

The wives were then asked if their husbands had changed in their relationship to them since the treatment. Thirteen LSD wives reported favorable changes, mostly in terms of more consideration and tolerance—one said it was just like when they first got married. One wife said there was no change in their relationship, her husband had always been pleasant to her, the other that while at times her husband would be affectionate with her, his actions were always less impressive than his words. Six of the comparison wives reported no change in their husbands' attitude toward themselves, one reported change only in the beginning when she could talk to him without recrimination. Another also reported temporary improvement in that her husband talked more with her when he first came home and was more understanding of their child. The other three reported more permanent gains, including increased affection, better communication, fewer complaints.

The wives were also asked whether they felt they themselves had changed in relation to their husbands. All but two of the LSD wives reported they felt they had changed. Most of the changes were positive; one reported she fell in love with her husband again, another that she had more respect for her husband because she felt he deserved it, another that she could now trust her husband completely.

However, one wife, though she felt her husband had improved, saw herself as more nagging and crabby than ever; another felt that having had a touch of independence and freedom while her husband was gone, she did not want him around; and a third felt that she still harbored bitterness which her husband expected her to laugh off and forget, and that she tried to keep things in rather than hurt him. Two of the comparison group wives reported no change in themselves, four reported only temporary receptivity and hopefulness when their husbands first came home, while one reported herself puzzled by her husband's behavior, and another sometimes sorry for herself and unable to trust her husband. Three reported permanent positive change in terms of being

more confident of their husbands, contented and loving.

No marked change was reported by the LSD group with regard to sexual relations. Of the LSD wives, nine said there was no difference at all, five acknowledged better relations, and one said that relations were different but not improved. The comparison group wives reported even less; eight saw no change in husbands, one saw only a temporary improvement, one found her husband interested but fearful, and one said things were much better.

When the wives were asked whether they blamed or had ever blamed themselves for their husbands' alcoholism, the LSD group expressed more sensitivity to their own involvement, or even guilt, in the problem than did the comparison group, and this in ways which they seemed to feel might yield to therapy for themselves.

The LSD wives were also asked what they had read or heard about LSD treatment before their husbands took it. Seven reported having read unfavorable things only, four had read or heard favorable things and unfavorable things, and the others had heard nothing about it. Quite certainly the wives did not feel that this would have miraculous effects upon their husbands. Nevertheless, asked whether they thought the treatment as a whole really helped their husbands, 13 in the LSD group answered with enthusiasm that it did, the other two responding with more caution. All but these two said they were glad their husbands had taken the treatment, and only one would not like her husband to take the drug again under any conditions.

In contrast, five of the 11 in the comparison group said treatment had helped their husbands, five that it helped somewhat, and two that it did not help at all.

Finally, both groups were asked if they had any questions, problems, or suggestions arising from treatment of their husbands. Response here in the LSD group was almost double that in the comparison group. While the content of the responses in the two groups were similar, the LSD group seemed to feel freer to talk, had more to talk about, and expressed themselves more vigorously.

The wives in both groups felt almost unanimously "left out" of the whole rehabilitation process. They would like to have been further enlightened on the subject of

alcoholism; they would like to have had an opportunity to talk to the psychiatrist about their husbands and about the treatment; seven in the LSD group would have liked concomitant treatment for their own problems; and virtually all in the LSD group felt they should have been prepared in some rather concrete ways for the husbands' homecomings. "We didn't know what to expect" was repeatedly expressed by these women.

Comment

This wives' eye view of the effects of LSD treatment of alcoholics strongly suggests a marked disparity between immediate and longer-run improvement. According to their wives, when the LSD-25 treated husbands first returned from the hospital they had changed markedly, particularly in areas of understanding, awareness, and tolerance of others. However, it would appear that after six months this treatment gain was considerably diminished, and the residual improvement consisted mainly in the absence of previous hostile and abusive behavior.

These findings confirmed those in our studies of the men themselves (reported elsewhere, as mentioned earlier³¹), for a marked favorable attitudinal change was revealed in pre- and posttreatment questionnaire measures while the men were still in the treatment center. Also, our follow-up studies of sobriety showed decided improvement at the three months and a sharp drop-off at the six-month period.

The pattern of immediate improvement and subsequent decline appeared also in the comparison group. However, the wives' ratings suggested the LSD husbands changed much more initially than the comparison group husbands, who had altered at that time mostly in terms of loss of negative traits, and at the six-month follow-up the LSD husbands still showed a marked advantage over the comparison group husbands.

Our study also strongly suggests secondary effects of LSD treatment on the spouses of the treated alcoholics. The wives of the LSD group saw themselves immediately after treatment as less discouraged, irritable, and resentful and more hopeful, considerate, and loving, though improvement was greatly diminished at six months. The comparison group wives also saw initial im-

provement in themselves following their husbands' treatment, but this had practically disappeared by six months. However, both initially and later, the improvement seen in themselves by the LSD group wives was much greater than that seen in themselves by the comparison group wives.

In general, the wives of both the LSD and comparison group saw much more positive than negative change in their husbands and felt that few problems had arisen as a result of the treatment. In the comparison group, the problems related mainly to feelings of stigma on the part of the husbands for having to be hospitalized for their alcoholism. In the LSD group, one of the problems at least, related to the particular group method used in the program: one wife complained that her husband began to drink when another LSD group member with whom he had developed a close attachment began to drink. In subsequent work with LSD, we have reduced the closeness of the treatment group, which we had hoped originally would reinforce good behavior but which we found also made adverse happenings contagiously discouraging.

Another wife felt that her husband was overconfident, that he looked upon the LSD treatment as a sort of miraculous cure-all, and his subsequent fall from sobriety justified her uneasiness. In subsequent work we took care to underline that this was not a miraculous cure, a point which we mistakenly felt we had covered adequately before.

It was said by another wife that after the LSD her husband's drinking behavior worsened. This was a possible negative effect of the drug; however, it may have been occasioned by the progressive nature of his illness. Further investigation of this matter would be necessary to find out whether in fact LSD treatment does make drinking behavior worse. Certainly this was mentioned by none of the other wives.

Another difficulty emerging from the treatment was that of the husband who, according to his wife, was more aware that he was no good. Of course, this could be regarded as a step forward, unless the husband used this as an excuse to drink.

The complaint of a fifth wife reflected the sort of problem which had led us to undertake the study in the first place. This was the woman who complained of her husband's increasing withdrawal from her and the

family. Interestingly enough, this was the one alcoholic in the LSD group who remained completely sober throughout the one-year follow-up period. However, as a result of years of marital difficulties, this wife was bitter, resentful, and hostile towards her husband and found it difficult to give up these feelings despite the marked improvement he was showing. Had she received better guidance, she might have managed to capitalize on her husband's improvement instead of seeing it increase the distance between them.

Such difficulties because of successful LSD treatment we had encountered in our follow-up of previous groups. As we mentioned earlier, one wife had complained that it was difficult for her to deal with her husband *because* he no longer acted like an adolescent, another that her husband was spending too much time on community activities. Another wife from an earlier series complained that after his LSD treatment her husband, who had previously wanted her to manage their finances, began to take over in this area, and that she found this hard to bear.

These problems related to successful treatment appeared to reflect an inability of the wives to shift from an adjustment appropriate to their husbands' lower level of function while drinking, and we felt they might be reduced by prior preparation of both husbands and wives with regard to what sort of changes each might have undergone as a result of the alcoholism and what changes might occur as a result of the treatment. Others working in the treatment of alcoholics have recommended working with the wives,³² but such preparation and advice would probably be specially important in the case of LSD treatment because of its dramatic effects. That more preparation of the wives and concurrent help and advice would be desirable, particularly in the case of LSD treatment, was brought out by the general comments and criticisms of both groups of wives with regard to the treatment programs.

In general, the reaction of the wives to the LSD treatment of their husbands was highly favorable and more enthusiastic than that of the comparison group wives; however, both groups expressed a great need to have better information about what went on in treatment and also guidance with re-

gard to how to help their husbands when they returned home from the hospital. The LSD wives were specially vociferous in their requests for concurrent help.

In conclusion, we felt that this exploratory study strongly suggested change of a dramatic sort in the LSD-treated alcoholics in the period immediately following treatment, change which should have made it easier for them to establish good relations with their families and communities. The alcoholic, while drinking, sinks deeper and deeper into a morass of self-pity, bitterness, resentfulness, and selfishness and this process had evidently been reversed by the LSD treatment. Even at the six-month period some improvement was sustained, for the wives reported their husbands then to be less bitter, resentful, etc. However, it was evident that while the wives also were initially favorably affected by their husbands' improvement, as in the case of the husbands, this improvement was soon diminished.

These findings underlined for us the crucial significance of preparing treated alco-

holics and their spouses for the period after LSD treatment and continuing to work with both in order to consolidate treatment gains.

In a study now in progress we have included the wives of alcoholics treated with LSD in the treatment program from the time the men first volunteer for the treatment. Individual and group sessions with the wives are held in which information with regard to the nature of alcoholism, LSD treatment, changes they might find in their husbands when they return home, and how to help maintain improvement is given. Wives are allowed to spend a few hours with their husbands while they are in the later hours of the LSD experience if both so desire. Also, after the husbands leave the treatment center, both husbands and wives are invited back for group meetings once a month and continuing support and advice are offered. Follow-up studies at three, six, and 12 month intervals will enable us to compare the effectiveness of this approach with that taken in previous work.

REFERENCES

1. Belden, E., and Hitchen, R.: The Identification and Treatment of an Early Deprivation Syndrome in Alcoholics by Means of LSD-25, *Amer J Psychiat* 119:986-986, 1963.
2. Benedetti, G.: Example of the Study of the Mental Structures and of the Pharmacodynamic Investigation of a Case of Alcoholic Hallucinosis, Character Neurosis and Psychoreactive Hallucinosis, *Acta Psychotherap Med Psychol* 1:176, 1951.
3. Bureau on Alcoholism, Regina, Saskatchewan: Apparent Results of Referrals of Alcoholics for LSD Therapy, mimeographed, December 1962.
4. Chwelow, H., et al: Use of LSD-25 in the Treatment of Chronic Alcoholism, *Quart J Stud Alcohol* 20:577-590, 1959.
5. Ditman, K.S.: The Use of LSD in the Treatment of the Alcoholic, read before the New York Medical Society on Alcoholism, New York, November 1962.
6. Fox, R.: Is LSD of Value in Treating Alcoholics? read before the Second Conference on Use of LSD in Psychotherapy, Amityville, Long Island, May 8-10, 1965.
7. Hoffer, A.: Remarks at 1959 conference on use of LSD in psychotherapy held under the auspices of the Josiah Macy, Jr. Foundation.
8. Jensen, S.E.: A Treatment Program for Alcoholics in a Mental Hospital, *Quart J Stud Alcohol* 23:243-251, 1962.
9. MacLean, J.R., et al: The Use of LSD-25 in the Treatment of Alcoholism and Other Psychiatric Problems, *Quart J Stud Alcohol* 22:34-45, 1961.
10. O'Reilly, P.O., and Reich, G.: Lysergic Acid and the Alcoholic, *Dis Nerv Syst* 23:331-334, 1962.
11. Osmond, H.: A Review of the Clinical Effects of Psychomimetic Agents, *Ann NY Acad Sci* 66:418-434, 1957.
12. Ramsay, R.; Jensen, S.; and Sommer, R.: Values in Alcoholics After LSD-25, *Quart J Stud on Alcohol* 3:443-448 (Sept) 1963.
13. Rolo, A.; Krinsky, L.W., and Goldfarb, L.: LSD as an Adjunct to Psychotherapy With Alcoholics, *J Psychol* 50:85-104, 1960.
14. Smith, C.M.: A New Adjunct to the Treatment of Alcoholism: The Hallucinogenic Drugs, *Quart J Stud Alcohol* 19:406-417, 1958.
15. Smith, C.M.: Exploratory and Controlled Studies of Lysergide in the Treatment of Alcoholism, *Quart J Stud Alcohol* 25:742-747, 1964.
16. Ballard, R.G.: The Interaction Between Marital Conflict and Alcoholism as Seen Through MMPI's of Marriage Partners *Amer J Orthopsychiat* 29:528-546, 1959.
17. Bensoussan, P.A.: Quelques caracteres particuliers des familles d'alcoholes et leurs consequences therapeutiques, *Rev Med Psychiat* 4:489-495, 1958.
18. Bullock, S.C., and Mudd, E.H.: The Interrelatedness of Alcoholism and Marital Conflict, *Amer J Orthopsychiat* 29:519-527, 1959.
19. Lemert, E.M.: Dependency in Married Alcoholics, *Quart J Stud Alcohol* 23:590-609, 1962.
20. Lewis, M.L.: The Initial Contact With Wives of Alcoholics, *Soc Casework* 35:8-14, 1954.
21. Mitchell, H.E., and Mudd, E.H.: Development of a Research Methodology for Achieving the Cooperation of Alcoholics and Their Non-Alcoholic Wives, *Quart J Stud Alcohol* 18:649-657, 1957.
22. Mitchell, H.E.: The Interrelatedness of Alcoholism and Marital Conflict, *Amer J Orthopsychiat* 29:547-559, 1959.
23. Price, G.M.: A Study of the Wives of Twenty Alcoholics, *Quart J Stud Alcohol* 5:620-627, 1945.
24. Whelan, T.: Wives of Alcoholics, *Quart J Stud Alcohol* 14:632-641, 1953.
25. Bailey, M.B.: Alcoholism and Marriage, *Quart J Stud Alcohol* 22:81-97, 1961.
26. Jackson, J.K.: "Alcoholism and the Family," in *Society, Culture and Drinking Patterns*, D. J. Pittman and C. R. Snyder (eds.), New York: John Wiley & Sons, 1962, pp 472-492.
27. Futterman, S.: Personality Trends in Wives of Alcoholics, *J Psychiat Soc Work* 23:37-41, 1953.
28. Pixley, J.M., and Stiefel, J.R.: Group Therapy Designed to Meet the Needs of the Alcoholic's Wife, *Quart J Stud Alcohol* 24:304-314, 1963.
29. Jackson, J.K.: The Adjustment of the Family to the Crisis of Alcoholism, *Quart J Stud Alcohol* 15:562-586, 1954.
30. Spencer, A.M.: Modifications in the Technique of LSD Therapy, *Comprehen Psychiat* 5:232-251, 1964.
31. Osmond, H., et al: Some Problems in the Use of LSD-25 in the Treatment of Alcoholism, read before the Second Conference on Use of LSD in Psychotherapy, Amityville, Long Island, May 8-10, 1965.
32. Connor, R.C.: *The Self-Concepts of Alcoholics*. D.J. Pittman and C.R. Snyder (eds.), New York: John Wiley & Sons 1962, pp 455-467.